



LESSONS LEARNED
WAH4HOSPITAL (WAH4H)
Asia Pacific College
2026

INTRODUCTION

The Lessons Learned document captures key insights from the WAH4H project to improve future academic and real-world software deployments.

LESSONS LEARNED APPROACH

Lessons were identified by the PM during post-sprint retrospectives and consolidated at project closure. They cover scope, cost, UAT, and risk management practices.

LESSONS LEARNED FROM THIS PROJECT

Lesson ID	Area	What Happened	Impact	Recommendation
LL-001	Scope confirmation	Scope was formally confirmed mid-Sprint 3 via CR-001 after significant documentation and WBS work had already begun.	Caused Sprint 3 replanning and increased workload.	Schedule client scope confirmation before large execution-phase expansion.
LL-002	Cost baseline	Flat ₱500/hr rates were used initially, conflicting with role-based EVM reporting later.	Required retroactive correction across multiple sections.	Use role-based rates from project start and document the source of each rate.
LL-003	UAT planning	Real UAT across 11 modules required significantly more re-testing effort than originally planned.	Sprint 3 actual hours (500.50h) far exceeded planned (243.50h).	Increase UAT and re-testing reserves for future healthcare systems.
LL-004	Risk alignment	Risk summaries across initiation and planning sections became inconsistent	Created internal document conflicts.	Maintain one risk source of truth and update cross-document summaries after each risk revision.



		after RMP v2 upgrade.		
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LESSONS LEARNED KNOWLEDGE BASE / DATABASE

These lessons are documented in the project closure report and shared with the APC Faculty Adviser for future cohort reference.

Lesson ID	Area	Knowledge Base Entry	Distribution
LL-001	Scope confirmation	Confirm client scope formally before large documentation or WBS phases begin. Log via formal Change Request.	Project Closure Report; APC Faculty Adviser; future Pseudoers-affiliated teams
LL-002	Cost baseline	Use role-based hourly rates derived from market benchmarks (JobStreet PH, NodeFlair PH, PayScale PH) from project start. Document rate source per role.	Project Closure Report; APC Faculty Adviser; future Pseudoers-affiliated teams
LL-003	UAT planning	Budget at least 2x planned hours for UAT re-testing in multi-module healthcare systems. Apply severity triage before committing to full re-testing cycles.	Project Closure Report; APC Faculty Adviser; future Pseudoers-affiliated teams
LL-004	Risk alignment	Maintain a single RMP version as the source of truth. After each RMP revision, update all cross-document risk summaries (Charter, Business Case, Feasibility Study) before distribution.	Project Closure Report; APC Faculty Adviser; future Pseudoers-affiliated teams

LESSONS LEARNED APPLIED FROM PREVIOUS PROJECTS

No formal lessons learned database from prior cohorts was available. Lessons from this project will serve as the baseline for future Pseudoers-affiliated teams.

PROCESS IMPROVEMENT RECOMMENDATIONS

Adopt role-based cost estimation from project start. Formalize scope confirmation as a required gate before Sprint 3 documentation begins. Build UAT reserves of at least 2x planned hours for multi-module healthcare systems. Maintain a single RMP version as the source of truth for all risk references across the master document.

#	Recommendation	Applies To	Priority
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1	Adopt role-based cost estimation from project start using published Philippine IT market benchmarks (JobStreet PH, NodeFlair PH, PayScale PH). Document the source of each hourly rate.	Cost Management Plan, EVM baseline	High
2	Formalize scope confirmation as a required gate event before Sprint 3 documentation phases begin. No large document batch production starts without a signed scope confirmation from the client.	Scope Management Plan, Change Control	High
3	Build UAT and re-testing effort reserves of at least 2× planned hours for multi-module healthcare systems. Apply defect severity triage to focus re-testing on critical and high items first.	Quality Management Plan, Schedule Baseline	High
4	Maintain a single RMP version as the source of truth for all risk references across the master document. After each RMP revision, update all dependent documents (Charter, Business Case, Feasibility Study) before distribution.	Risk Management Plan, Configuration Control	Medium

Sponsor Acceptance

Approved by the Project Sponsor:

Sir Pey Canlas
Executive Director, Wireless Access for Health (WAH)

Date: _____